



Olanzapine Pamoate Long-Acting Injection: prescribing, administering and monitoring

1. Background

1.1. About this document

This guideline provides guidance to staff for the prescription, administration and monitoring of olanzapine pamoate long-acting injection (LAI).

1.2. Key definitions

Term	Definition
Consent (to medical treatment)	In the context of health care, consent is a consumer's agreement that a health practitioner can proceed to perform a specific proposed treatment.
Verbal consent	Where a verbal (compared with written) agreement to a treatment or procedure is discussed with and agreed to by a consumer. Verbal consent does not meet the requirements for explicit consent.
Long-Acting Injection	Intramuscular (IM) antipsychotic injections with a prolonged effect resulting in the slow release of the active principle or the continued absorption of small amounts of the dosage of the medication over an extended period.
Olanzapine pamoate (Zyprexa[®] Relprevv[®])	A long-acting, injectable form of olanzapine used for antipsychotic maintenance treatment of schizophrenia. It is not to be confused with the short acting olanzapine injection (Zyprexa[®] IM) which is used for acute agitation and arousal. Olanzapine pamoate LAI is a crystalline salt formulation suspended in water. When injected into the muscle the crystals slowly dissolve and dissociate releasing olanzapine. Olanzapine pamoate comes in the following strengths: • Olanzapine pamoate monohydrate 210mg long-acting injection • Olanzapine pamoate monohydrate 300mg long-acting injection • Olanzapine pamoate monohydrate 405mg long-acting injection.

2. Guidelines

2.1. Indications for use

Olanzapine pamoate LAI is indicated for adult consumers:

- requiring maintenance treatment of schizophrenia after being sufficiently stabilised during acute treatment with oral olanzapine
- with demonstrated tolerability and response to oral olanzapine
- willing and able to adhere to the required post injection monitoring period
- who consent to not driving or operating machinery for the remainder of the day of injection
- of community based mental health teams able to complete the required post injection monitoring after each administration and the facilities/capacity to manage olanzapine toxicity including transfer of the consumer to appropriate emergency services in the event of PIS

2.2. Precautions for use

Olanzapine is classed as Category C¹ by the Australian categorisation system for prescribing medicines in pregnancy. There are no adequate and well-controlled studies in pregnant women. Consumers should be advised to notify their treating team if they become pregnant or intend to become pregnant during treatment¹.

Consumers should be advised not to breastfeed if they are receiving Olanzapine pamoate LAI¹.

Olanzapine pamoate LAI must not be commenced if clinics are unable to accommodate post injection monitoring requirements.

2.3. Prescribing

Olanzapine pamoate LAI must be prescribed in accordance with the manufacturer's product information and [PBS requirements](#). Prescribing outside of these limits constitutes off label prescribing and may require an individual patient approval (IPA).

Prescriptions for olanzapine pamoate must be documented on the Intramuscular Long-Acting Injection Chart.

Table 1: Initiating Olanzapine Pamoate LAI

Total oral olanzapine dose	Recommended starting dose of olanzapine pamoate (for 8 weeks)	Maintenance dose after 8 weeks of starting olanzapine pamoate treatment
10mg per day	210mg every 2 weeks or 405mg every 4 weeks	150mg every 2 weeks or 300mg every 4 weeks
15mg per day	300mg every 2 weeks	210mg every 2 weeks or 405mg every 4 weeks
20mg per day	300mg every 2 weeks	300mg every 2 weeks

2.4. Consumer education and consent

Prior to commencement of olanzapine pamoate LAI the consumer must be provided with adequate education and information including:

- verbal and written information on olanzapine pamoate that is appropriate in terms of the consumers language and communication needs, health literacy, and culture. Choice and Medication is the recommended written resource
 - ask all Aboriginal consumers if they would like an Aboriginal Health Liaison Officer (AHLO), Aboriginal Mental Health Liaison (AMHL) or Aboriginal Health Practitioner (AHP) involved in education, yarning about treatment options, and the consent process. Document the offer and the consumer's decision in the consumers medical record.
- the risk of PIS, the post injection monitoring requirements, including the potential for the duration of post injection monitoring to be extended, and actions that may be required if PIS occurs including the potential transfer to an emergency department for ongoing care.
- alternative treatment options available and rationale for the choice of olanzapine pamoate LAI
- not to drive or operate heavy machinery for the remainder of the day
- the opportunity to ask questions and be heard and afforded time and support to understand the information presented where practicable

Informed consent is required and should be obtained:

- through a process of clear communication, discussion and shared decision making
- with involvement of personal support persons in treatment planning decisions wherever possible
- with staff documenting the consumer's verbal consent to olanzapine pamoate LAI treatment in the consumer's medical record

For consumers subject to a Community Treatment Order (CTO):

- where olanzapine pamoate LAI is specified as a condition of the CTO, informed consent should be sought and documented wherever possible but is not a mandatory requirement for administration

2.5. Prior to administration

Confirm that voluntary consumers have provided informed consent to both the administration of the injection and compliance with the post-injection monitoring requirements. Document this consent in the consumer's medical record.

If the consumer states they are unwilling to remain for the observation period, contact the treating Psychiatrist, Health Service Medical Practitioner (HSMP) or registrar for further advice:

- Olanzapine pamoate LAI should **not** be administered where the required post-injection monitoring cannot be completed, due to the risk of post-injection syndrome.
- In **exceptional circumstances**, administration may proceed following a documented risk-benefit assessment by the treating team medical officer, considering the risks of incomplete

monitoring, the risks of delaying or ceasing treatment, and alternative treatment options. Where administration does proceed, the clinical rationale **must** be clearly documented in the consumer's digital medical record.

Ensure the consumer has transport arranged from the clinic and agrees not to drive or operate heavy machinery for the remainder of the day.

Assess for signs of intoxication with alcohol or other drugs on presentation. If the consumer presents as intoxicated, contact the treating Psychiatrist, HSMP or registrar for further advice before administering olanzapine pamoate LAI.

Complete the '*Prior to Administration*' section of the SMHS Olanzapine Pamoate Post Injection Checklist.

2.6. Reconstitution

Use standard aseptic techniques for the reconstitution of parenteral products.

It is recommended that gloves are worn during the reconstitution process as olanzapine pamoate may be irritating to the skin. Flush with water if contact is made with the skin.

Reconstitute as per manufacturer's instructions below using the diluent, syringe and needles provided. Once reconstituted as per Table 2, the resulting olanzapine concentration is always 150mg/1mL.

Table Two: Reconstituting Olanzapine Pamoate LAI

Prescribed dose	Zyprexa Relprevv [®] vial strength (mg)	Volume of diluent to be added	Volume to be injected
150mg	210mg	1.3mL	1mL
210mg	210mg	1.3mL	1.4mL
300mg	300mg	1.8mL	2mL
405mg	405mg	2.3mL	2.7mL

Note: There will always be left over diluent and reconstituted suspension even after the appropriate dose has been removed.

Once reconstituted, the product should be used as soon as possible:

- the reconstituted product may be stored for up to 6 hours at room temperature (25°C) before being used

2.7. Administration

Administration may only take place in a healthcare facility such as an inpatient setting or community mental health clinic by staff with knowledge of symptoms of PIS.

Olanzapine LAI must **only** be administered by deep intramuscular (IM) GLUTEAL injection,

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alternating between the left and right sides. Administration in any other site is deemed off label and rationale must be documented in the consumer's medical record by the Consultant Psychiatrist:

- the deltoid has the greatest blood flow of any muscle routinely used for IM injections and is associated with higher risk of PIS.

Use the appropriate specified gauge safety needle that is provided by the manufacturer:

- for non-obese consumers: use 1½ inch (38mm) 19G needle
- for obese consumers: use the 2-inch (50mm) 19G needle

Note: the approved product information does not define the terms “non-obese consumers” and “obese consumers” therefore clinical judgement is required.

Prior to administration, aspirate the syringe:

- if blood is observed in the needle or syringe, do **not** administer the dose
- withdraw the needle, discard the syringe, needle and product, and prepare a new dose for administration

2.8. Post injection syndrome and monitoring requirements

Due to the risk of PIS, consumers are required to remain under observation for a period post administration of olanzapine pamoate LAI. The time to onset of initial signs and symptoms is within one hour for 91% of cases. Full recovery from PIS usually occurs within 24-72 hours².

Monitoring observations for PIS includes assessment of:

- level of alertness
- orientation to person, time and place
- physical symptoms e.g. dizziness, dysarthria, ataxia.

Monitoring observations must be:

- **completed every 30 minutes for a minimum of 2 hours**
- extended to 3 hours or longer post injection dependent on signs and symptoms and medical advice
- documented on the SMHS Olanzapine Pamoate Post Injection Checklist.

Note: PIS may occur after **any** administration of olanzapine pamoate LAI, regardless of prior tolerance; therefore, the above monitoring is required after **every** administration.

Symptoms of PIS are consistent with olanzapine overdose and include sedation varying from mild to severe and delirium (disorientation, confusion, agitation, anxiety and other cognitive impairment), extrapyramidal symptoms, dysarthria (slurred speech), ataxia (staggering, uneven gait), aggression, dizziness, weakness, hypertension and convulsion.

Note: The same potential for PIS exists with every injection of olanzapine pamoate monohydrate.

If any symptoms of PIS are noted, immediately escalate to nursing and medical for review and commence physiological observations every 15 minutes and document on the Adult Observation and Response Chart. Staff will activate a MET call via local processes when clinically indicated.

Refer to:

- [FSFHG Recognising and responding to acute deterioration](#)
- [RkPG Recognising and responding to the deteriorating patient](#)

Document any adverse event related to administration of olanzapine pamoate LAI as a clinical incident in the Datix Clinical Incident Management System (Datix CIMS). It is also strongly recommended to report the incident to the TGA (via the TGA Adverse Event Management System ([AEMS](#)))

2.9. Non-adherence to post monitoring requirements

Consumers who express a desire to leave prior to the completion of the required post injection monitoring period will have their care escalated by the following process:

- encourage consumer to stay for as long as possible. PIS is most common within the first 1-hour post administration
- reinforce the reasons for monitoring and the risks of leaving prior to the end of the monitoring period
- engage with care coordinator, AMHL, AHLO or AHP as relevant
- escalate to medical officer/senior clinician for review
- confirm consumer's best contact number, next of kin details and obtain information on the location the consumer will be heading to after leaving.

Where the consumer leaves prior to the completion of the post injection monitoring period:

- request the consumer sign the FSFHG Olanzapine Pamoate Post Injection Checklist
- contact the consumer's carer, personal support person, GP or local Aboriginal Medical Service and inform them of the situation and post injection monitoring requirements
- staff responsible for completing post injection monitoring will attempt to contact the consumer by phone **every half an hour for 2 hours** after the injection
- if the consumer is unable to be contacted, contact the consumers carer or personal support person. If unable to contact consumer or carer, consider completing a community home visit in discussion with the multidisciplinary team
- document any observations obtained and actions taken after the consumer declined to remain for further monitoring in the consumer's digital medical record

The treating consultant psychiatrist will review the suitability of continuing olanzapine pamoate LAI and develop a plan to ensure compliance prior to the next planned administration date.

For consumers who are repeatedly non-adherent with post-injection monitoring requirements, the treating Consultant Psychiatrist **must** document a clear clinical justification for continuing olanzapine pamoate LAI in the consumer's digital medical record. This justification must address the risks of continuing treatment in the context of non-adherence to monitoring, the risks of ceasing treatment, and alternative treatment options considered and the reasons they are not being pursued. This decision and rationale **must** be reviewed and re-documented prior to each administration.

3. Compliance Monitoring

Each SMHS MH site Service Director/Co-Director and Heads of Service are to ensure compliance with this policy. Compliance monitoring methods include the following:

- Auditing of monitoring requirements conducted as per the SMHS Mental Health Audit Schedule.
- Nurse Unit Managers (NUM)/ Team Leaders routine monitoring of clinical incident trends in relevant areas.

4. Legislation

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

- [Mental Health Act 2014](#)

5. Related Documents

5.1. Department of Health policies

- [MP 0175/22 Consent to Treatment](#)
- [MP 0122/19 Clinical Incident Management](#)
- [MP 0181/24 Safety Planning for Mental Health Consumers](#)
- [Aboriginal Cultural eLearning \(ACeL\)](#)
- [MyLearning - \[SMHS\] Aboriginal Person Centred Care Training](#)
- [WA Aboriginal Health and Wellbeing Framework 2015-2030](#)

5.2. SMHS policies

- [Consent to Treatment](#)
- [Hand hygiene and skin integrity](#)

5.3. Other documents

- [Chief Psychiatrists Standards for Clinical Care](#)
- [Chief Psychiatrists Clinicians Practice Guide to the Mental Health Act 2014](#)

5.4. References

1. Therapeutic Goods Administration. Zyrexa Relprevv (Olanzapine pamoate monohydrate) powder for injection Australian Product Information. Updated October 2025. Accessed December 10, 2025.
<https://www.ebs.tga.gov.au/ebs/picmi/picmirepository.nsf/pdf?OpenAgent=&id=CP-2024-PI-01648-1&d=20251210172310101>

2. Government of New South Wales. Safety Notice 016/21: Identification and Monitoring of Post-Injection Syndrome Olanzapine Pamoate Long-Acting Injection. 2021. Available from [2021-sn-016.PDF \(nsw.gov.au\)](https://www.nsw.gov.au/health-and-medical-services/safety-notice-016-21-identification-and-monitoring-of-post-injection-syndrome-olanzapine-pamoate-long-acting-injection)

5.5. Version Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP V1	2 April 2026	27 March 2026	27 March 2029	March 2029	Original version

5.6. Approval

Approval by	Executive Director Mental Health
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This document can be made available in alternative formats on request.

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